



# Breastfeeding Policy

## 1. Scope

| Site                   | Operational Area | Applicable to         |
|------------------------|------------------|-----------------------|
| Armada Kalamunda Group | Obstetrics       | Midwifery and Medical |

## 2. Purpose

The purpose of this policy is to ensure that all staff at Armada Kalamunda Group (AKG) understand their role and responsibilities in supporting expectant and new mothers and their partners to feed and care for their baby in ways which support optimum health and well-being.

AKG recognises breastfeeding as the biological and social norm for infant and young child feeding and supports the importance of creating and delivering a health care environment, where breastfeeding is encouraged, promoted, and supported by all staff to parents.

AKG recognises that breastfeeding is an unequalled way of providing ideal food for the healthy growth and development of infants and is beneficial to mothers, families, and society.

This policy is based on the 10 Steps to Successful Breastfeeding and the recommendations of [Baby Friendly Hospital Initiative](#), UNICEF, and the World Health Organisation (WHO, 2018). To Ensure consistent, comprehensive, evidence based breastfeeding information and support is provided to parent by all health care professionals working at EMHS.

## 3. Policy statement

Armada Kalamunda Group is committed to:

- Providing the highest standard of care to support expectant and new mothers and their partners to feed their baby and build strong and loving parent- infant relationships. This is in recognition of the profound importance of early relationships to future health and well-being, and the significant contribution that breastfeeding makes to good physical and emotional health outcomes for children and mothers.
- Ensuring that all care is mother, and family centred, non-judgemental and that mothers receive clear and impartial information to enable them to make a fully informed choice as to how they feed and care for their babies after birth. Infant feeding decisions are supported and respected.
- Working together across disciplines and organisations to improve mothers' / parents' experiences of care.
- Staff members wishing to return to work while continuing to breastfeed are entitled to flexible lactation breaks and management support. Refer to the [EMHS Employee Breastfeeding Policy](#).

## 4. Breast milk substitute feeding

Parents who have made a fully informed choice to feed their babies with breastmilk substitute are to be shown how to prepare breastmilk substitute feeds correctly, on an individual basis in the postnatal period. No group instruction will be provided, and parents will be supported in their decision.

### 4.1 The Baby Friendly Health Initiative (BFHI) overview

BFHI is a joint World Health Organisation (WHO) and United Children's Fund (UNICEF) project. The aim is to give every baby the best start in life by creating a health care environment where breastfeeding is the norm and practices known to promote the health and well-being of all babies and their mothers are followed.

The ten steps to successful breastfeeding are the global standard by which healthcare facilities are assessed and accredited. A 'baby friendly' facility is one where mother's informed choice of feeding is encouraged, respected, and supported.

## 5. Principles

All staff of AKG are required to adhere to the AKG Breastfeeding Policy to avoid conflicting advice. Any deviation from the policy must be justified and recorded as a variance of care in the mother and/or baby's healthcare record. The policy is directed to mothers of healthy babies born at term. If a baby is unwell or pre-term, it may require a comprehensive feeding plan, in collaboration with the baby's paediatrician, general practitioner, lactation consultant, midwife, aboriginal health liaison officer and aboriginal grandmother.

The Policy principles reflect the key elements of the 'Ten Steps to Successful Breastfeeding'.

Step 1a: Comply fully with the International Code of Marketing of Breast-milk Substitutes and relevant World Health Assembly resolutions.

Step 1b: Have a written infant feeding policy that is routinely communicated to staff and parents.

Step 1c: Establish ongoing monitoring and data-management systems.

Step 2: Ensure that staff have sufficient knowledge, competence, and skills to support breastfeeding.

Step 3: Discuss the importance and management of breastfeeding with pregnant women and their families.

Step 4: Facilitate immediate and uninterrupted skin-to-skin contact and support mothers to initiate breastfeeding as soon as possible after birth.

Step 5: Supporting mothers to initiate and maintain breastfeeding and manage common difficulties.

Step 6: Do not provide breastfed newborns any food or fluids other than breast milk, unless medically indicated.

Step 7: Rooming-in - allow mothers and infants to remain together 24 hours a day.

Step 8: Support mothers to recognise and respond to their infants' cues for feeding.

Step 9: Counsel mothers on the use and risk of feeding bottles, teats, and pacifiers.

Step 10: Coordinate for timely access to ongoing support and care.

## 6. Aboriginal Health Impact Statement and Declaration

The voice of the Aboriginal community is reflected in the Breastfeeding policy through consultation with EMHS Aboriginal Health Strategy Team to Ensure that cultural appropriateness and the health impacts on Aboriginal people have been considered and incorporated.

**Note: Within Western Australia, the term Aboriginal is used in preference to Aboriginal and Torres Strait Islander, in recognition and Aboriginal people are the original inhabitants of Western Australia. No disrespect is intended to our Torres Strait Island colleagues and community.**

## 7. Legislative context

The following documents are required to give effect to this policy:

- Health Department of Western Australia (HDWA) [Consent to Treatment Policy MP 0657/16](#)
- HDWA [Safe Infant Sleeping Policy MP 0106/19](#)
- East Metropolitan Health Service (EMHS) [Patient Identification and Procedure Matching EMHS: 21](#)

## 8. Supporting information

The following documents support the implementation of this policy:

- World Health Organization (WHO) [Baby-friendly Hospital Initiative](#)
- [Neonatal Feeding: Fortification of Breast Milk Guideline PAE-GUI-0280-10](#)
- [Neonatal Feeding: Cleaning and Sterilising Breast Pump Equipment Procedure PAE-PRO-0281-17](#)
- [Neonatal Ingestion of Breast Milk from an Incorrect Donor Policy AKG-POL-0285-16](#)
- [Neonatal Feeding: Expressed Breast Milk – Identification, Storage and Thawing Guideline PAE-GUI-0282-16](#)
- [Safe Infant Sleeping Policy](#)
- [Safe Infant Sleeping Guideline](#)
- AK MR 73.8.1 Expressing with an electric breast pump for a baby in NNU
- AK MR 73.8.2 Vulnerable Baby
- AK MR 73.8.3 Blocked Milk Ducts
- AK MR 73.8.4 Inverted/flat nipples
- AK MR 73.8.5 Low breastmilk supply
- AK MR 73.8.6 Mastitis
- AK MR 73.8.7 Nipple trauma
- AK MR 73.8.8 Sleepy newborn
- AK MR 73.8.9 Weight loss beyond 10%
- AK MR 73.8.10 Breast Abscess

## 9. Compliance, monitoring and evaluation

Compliance with this policy will be monitored by the Midwife Manager and reported via the Obstetrics and Gynaecology Safety and Quality Departmental meeting.

Breastfeeding variance auditing compliance will be undertaken 6 monthly with audit findings to be presented at the Clinical Outcomes Committee Meeting and the Obstetrics and Gynaecology Safety and Quality Departmental meeting.

## 10. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- National Safety and Quality Health Service (NSQHS) Standards (second edition):
- Standard 5 – Action 5.27 Nutrition and hydration
- The health service organisation that admits patients overnight has systems for the preparation and distribution of food and fluids that include nutrition care plans based on current evidence and best practice
- Standard 5 – Action 5.28
- The workforce uses the systems for preparation and distribution of food and fluids to:
  - a. Meet patients' nutritional needs and requirements
  - b. Monitor the nutritional care of patients at risk
  - c. Identify, and provide access to, nutritional support for patients who cannot meet their nutritional requirements with food alone
  - d. Support patients who require assistance with eating and drinking
- Standard 6 – Action 6.5 Correct identification and procedure matching
- The health service organisation:
  - a. Defines approved identifiers for patients according to best-practice guidelines
  - b. Requires at least three approved identifiers on registration and admission; when care, medication, therapy, and other services are provided; and when clinical handover, transfer or discharge documentation is generated.
- Standard 6 – Action 6.7 Clinical handover
- The health service organisation, in collaboration with clinicians, defines the:
  - a. Minimum information content to be communicated at clinical handover, based on best-practice guidelines
  - b. Risks relevant to the service context and the particular needs of patients, carers and families
  - c. Clinicians who are involved in the clinical handover
- Australian Breastfeeding Association: Expressing and storing breastmilk

## 11. References

1. World Health Organisation. (2020). Nutrition: Breastfeeding.  
[https://www.who.int/nutrition/topics/exclusive\\_breastfeeding/en/](https://www.who.int/nutrition/topics/exclusive_breastfeeding/en/)
2. World Health Organisation. (1981). International Code of Marketing of Breast-Milk Substitutes and subsequent resolutions,  
<https://www.who.int/nutrition/netcode/resolutions/en/>

3. World Health Organisation and UNICEF. (2018). Protecting, promoting, and supporting breastfeeding in facilities providing maternity and newborn services: the revised Baby-friendly Hospital Initiative 2018. <https://www.who.int/nutrition/publications/infantfeeding/bfhi-implementation/en/>
4. Womens and Newborn Health Service. (2022). Breastfeeding Policy. <https://healthpoint.hdwa.health.wa.gov.au/policies/Policies/NMAHS/WNHS/WNHS.PM.Breastfeeding.pdf>
5. Marketing in Australia of infant Formulas: Manufacturers and Importers Agreement, The MAIF Agreement (2015). <https://www.accc.gov.au/system/files/publicregisters/documents/D15%2B143530.pdf>
6. Australian National Breastfeeding Strategy 2019 and beyond. [Australian National Breastfeeding Strategy collection | Australian Government Department of Health and Aged Care](#)

## 12. Policy owner

Enquiries relating to this policy may be directed to:

### Director Nursing and Midwifery

Title: Nurse Unit Manager  
 Department: Maternity Unit  
 Email: Shannon.scari@health.wa.gov.au

## 13. Review

This policy will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

| Version | Effective from | Effective to | Amendment(s)                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                    |
|---------|----------------|--------------|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| V5.0    | 06/03/2025     | 06/03/2028   | Principles: Removal of explanation of Ten Steps to Successful Breastfeeding<br>Inclusion of "All staff of AKG are required to adhere to the EMHS Breastfeeding Policy to avoid conflicting advice. Any deviation from the policy must be justified and recorded as a variance of care in the mother and/or baby's healthcare record.<br>The policy is directed to mothers of healthy babies born at term. If a baby is unwell or pre-term, it may require a comprehensive feeding plan, in collaboration with the baby's paediatrician, general |

|  |  |  |                                                                                                                   |
|--|--|--|-------------------------------------------------------------------------------------------------------------------|
|  |  |  | practitioner, lactation consultant and midwife".<br>Breastfeeding variances:<br>updated list of current variances |
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## 14. Authorisation

This policy has been authorised and issued by the Director of Nursing and Midwifery.

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|---------------------------|-----------------------------------------------------|
| <b>Authorised by</b>      | Helen Fullarton – Director of Nursing and Midwifery |
| <b>Authorisation date</b> | 05/03/2025                                          |
| <b>Published date</b>     | 06/03/2025                                          |